Eastside Community Health Clinic AI use policy

Draft for board review · Effective when the board approves it · Owner: Not yet named (see open questions) · Next review: In 12 months, and after the scribe pilot ends

Why we have this policy. We want AI to lift routine charting and writing off your plate. We must protect patient privacy and keep people in charge of care.

Who it covers. Everyone who works or volunteers here, including contractors and board members.

TOOLS WE USE

- **AI scribe from our health record vendor:** Three-provider pilot with a signed business associate agreement; visit notes only, and it may hold patient information.
- **No other AI tool:** Nothing else is approved yet.
- **New tools, add-ons and AI features.** Ask first before adding any tool, plugin, agent or AI feature, even inside software we own. Approvers check privacy first.
- **Accounts.** Use your work account. Turn off any setting that lets the vendor train on what you type.

WHAT YOU CAN USE AI FOR

- Pilot providers: let the approved scribe listen to visits and draft the note.
- Pilot providers: edit the scribe's draft so it matches the visit.
- Pilot providers: start from the scribe's draft instead of a blank note, to finish charting sooner.

NEEDS APPROVAL FIRST

- Tool that could touch patient information (business associate agreement first) — *Nadia, privacy and security officer*
- Any clinical use of AI — *Dr. Okafor, medical director*
- Any other new tool or use — *Beth, chief operating officer*

OFF LIMITS

- **Never put this into an AI tool** unless the tool is approved for it above: Passwords and logins never go in. Nothing else goes in unless the tool is approved above. Covered by HIPAA: diagnoses, medications, behavioral health notes, HIV status, reproductive health, immigration status, insurance numbers. Also substance use treatment records (extra federal protection), income documents, staff files, donor lists.
- No patient information in any tool without a signed business associate agreement. No exceptions.
- Never let AI make a clinical decision. A clinician decides and signs.

CHECK EVERYTHING BEFORE IT GOES OUT

- Providers read every word of a scribe note before signing.
- Whoever sends AI-assisted work owns it. Check facts, names, numbers and tone; clinicians review patient handouts.

DECISIONS ABOUT PEOPLE

- AI may only draft or help someone look things up. People decide who gets care.
- A person decides sliding fee eligibility from the documents.

BEING OPEN ABOUT AI

- Tell patients when the scribe is listening. They can say no; use the verbal consent script.
- If we ever add a website chatbot, it must say it is not a person.

IF SOMETHING GOES WRONG

- Tell Nadia right away if patient information may have gone into an AI tool.
- Nadia decides if it is a breach and runs the HIPAA process. Reporting a mistake is expected, never punished.

WHO OWNS THIS POLICY

- The compliance committee reviews this policy; the board approves it and changes. Staff get it at hire and yearly.

OPEN QUESTIONS FOR THE BOARD

- After the pilot, do we roll the scribe out to all providers, and should behavioral health use it at all?
- May staff use ChatGPT, OpenEvidence, UpToDate AI search or Google Translate, with no patient information, and for which tasks?
- May AI suggest billing codes or help with Spanish patient materials, and who approves and checks that?
- What do we tell HRSA and state funders about AI use, and who owns this policy day to day?

Sample policy for an invented organization, drafted by Fulton Ring from a mock 30-minute call. Not legal advice; not reviewed by a real nonprofit.